CHAPTER 5

The Psychoanalytic Hiatus

Many histories of psychiatry see psychoanalysis as the end point of the story, the goal to which all previous events had been marching. Yet with the hindsight of half a century since Freud's death in 1939, we are able to achieve a different perspective, in which psychoanalysis appears not as the final chapter in the history but as an interruption, a hiatus. For a brief period at mid-twentieth century, middle-class society became enraptured of the notion that psychological problems arose as a result of unconscious conflicts over long-past events, especially those of a sexual nature. For several decades, psychiatrists were glad to adopt this theory of illness causation as their own, especially because it permitted them to shift the locus of psychiatry from the asylum to private practice. But Freud's ideas proved short-lived. In the longer perspective of history, it was only for a few moments that the patient recumbent upon the couch, the analyst seated silently behind him, occupied the center stage of psychiatry. By the 1970s, the progress of science within psychiatry would dim the lights on this scenario, marginalizing psychoanalysis within the discipline of psychiatry as a whole. In retrospect, Freud's psychoanalysis appears as a pause in the evolution of biological approaches to brain and mind rather than as the culminating event in the history of psychiatry.

Yet it was a pause of enormous consequence for psychiatry. Freud's psychoanalysis offered psychiatrists a way out of the asylum. The practice of depth psychology, based on Freud's views, permitted psychiatrists for the first time in history to establish themselves as an office-based specialty and to wrest psychotherapy from the neurologists. Moreover, psychiatrists aspired to a monopoly over this new therapy. In the mind of the public, psychotherapy and psychoanalysis became virtually synonymous. If patients wanted one of the fashionable new depth therapies they would have to go to a psychiatrist for it, for the American Psychoanalytic Association initially insisted that only

MDs could be trained as analysts, and later that only psychiatrists could be so. In retrospect, this insistence was bizarre, for psychoanalysis required no more medical training than astrology, and the attempt to impose a medical monopoly over Freud's technique was a self-interested ploy to exclude psychologists, psychiatric social workers, and other competitors from the newly discovered fountain of riches.

Ultimately, psychoanalytically oriented psychiatrists were unable to preserve their monopoly. After the 1960s, all manner of nonmedical types demanded admission to the training institutes, for there was no intrinsic reason why professors of English could not do analysis as well as psychiatrists. Even worse, what had previously passed for the scientific basis of psychoanalysis began to collapse. It could not be simultaneously true that one's psychological problems were caused by an abnormal relationship to the maternal breast and by a deficiency of serotonin. As evidence began to accumulate on the biological genesis of psychiatric illness, psychiatry began to regain the scientific footing it had lost at the beginning of the analytic craze: The brain was indeed the substrate of the mind. By the 1990s a majority of psychiatrists considered psychoanalysis scientifically bankrupt.

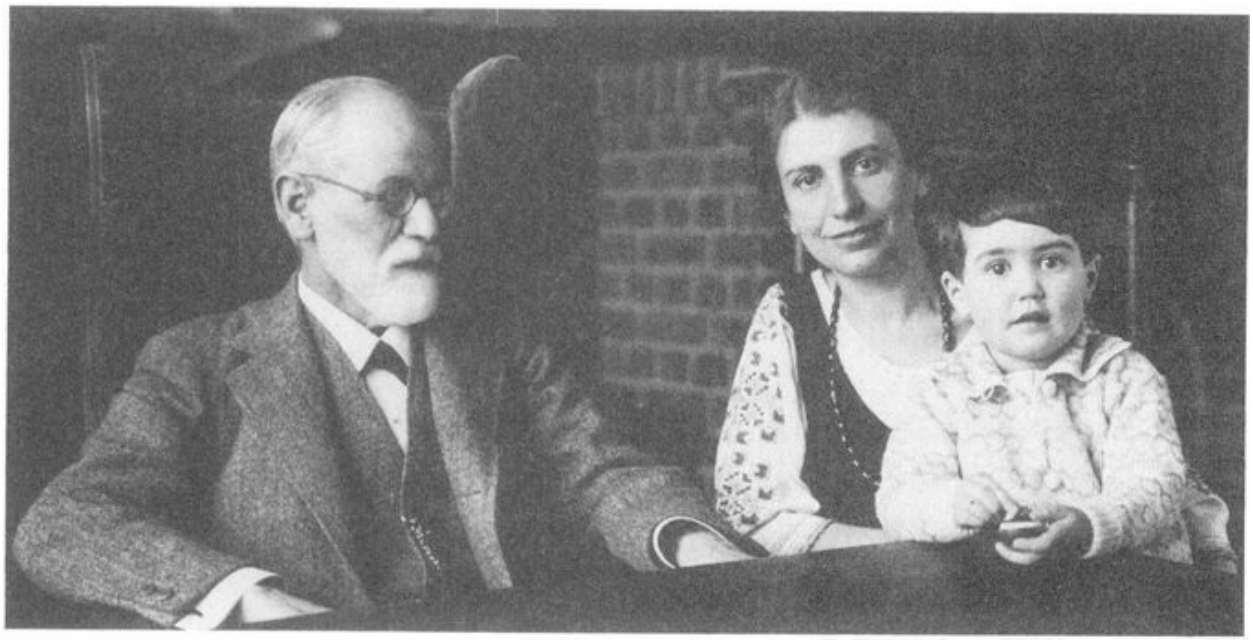
Thus Freud's model of the unconscious and the elaborate therapeutic techniques he devised for laying bare its supposed contents failed to stand the test of time. Accordingly, analysis largely vanished from psychiatry, discredited as a medical approach to the problems of mind and brain, although nonmedical psychoanalysis continued to flourish. The whole affair turned out to be the artifactual product of a distinctive era. Psychoanalysis failed to survive because it was overtaken by science, and because the needs that it initially met became dulled in our own time.

Freud and His Circle

Freud's psychoanalysis said that repressed childhood sexual memories and fantasies caused neurosis when reactivated in adult life. Such neurosis could be cured by an elaborate technique emphasizing dream analysis, free association, and the working through of a "transference-neurosis" (in which the analyst represents one of the patient's parents as a love-object, the patient then living out earlier childhood attitudes). Thus psychoanalysis was

intrinsically very psychiatric, in that it addressed disorders that doctor and patient agreed were disorders of the mind.

Sigmund Freud, (the founder of psychoanalysis) was born in 1856 and died in 1939. Here he is seen later in life with his daughter Anna Freud. The child is unidentified (courtesy of the Library of Congress, Prints and Photographs Division).



Yet ironically, the doctrine of psychoanalysis had its origin among nonpsychiatrists: neurologists, family doctors, and specialists in physical medicine whose patients craved some kind of caring, intimate, and ongoing contact with their physicians. The problem was that the plecebo therapies these doctors used, such as hydrotherapy, electrotherapy, or dietetic therapy, often failed to transmit the message that care was being conveyed as well as treatment. In psychoanalysis by its very nature, doctor and patient communicate in the enterprise of soul-searching, creating the suggestion that one is being cared for emotionally. Thus psychoanalysis became popular initially because it filled a sentimental gap in the consultation. It offered a doctor-patient relationship in which patients basked in what they believed to be an aura of concern.

Numerous physicians other than Freud understood these psychological

cravings, but Freud was the first to elaborate a therapy that would appeal to middle-class sensibilities, in particular to the desire for leisurely introspection. Yet his theories possessed a powerful additional resonance because, owing to his own ethnic origin and social position, he had privileged access to a group of patients who were especially needy in psychological terms: middle-class Jewish women in families undergoing rapid acculturation to West European values. The story of Freud's life is a familiar one.

Sigmund Freud was born in 1856 in the small town of Freiberg in Moravia, the son of merchant Jacob Freud and his third wife Malia. Four years later, the Freud family moved to Vienna, the city where Freud grew up and the centerpiece of the story of psychoanalysis. Though the Freuds arrived under somewhat reduced financial circumstances, they remained thoroughly middle class—never being without servants in the household for example—and the history of psychoanalysis would always reflect the experiences of people like the Freud family: educated, well-off, psychologically sensitive, and secularized.

Although by 1860 every city in Western Europe had a contingent of Jews, the Jews of Vienna were distinctive in constituting virtually the city's entire middle class. Whatever circle one examines—journalists, bankers, businesspeople, academics—all had a significant Jewish component by the end of the nineteenth century. This tremendous preponderance of Jews in the middle classes reflected the great social progress the Jews of Europe had made since the end of the eighteenth century, when they lived largely sequestered in the small towns of Poland, Russia, and the Ukraine. As a result of the Jewish emancipation of the nineteenth century, the small-town Jews of the east flocked to the cities of the west, using the high-school diploma as a launching pad for careers in the liberal professions. In 1890, for example, 33 percent of students at the Vienna University were of Jewish origin.[1] Fully one half of the professors of Vienna's medical faculty were Jewish.[2] As many as two-third's of the city's physicians were Jews.[3] Thus, rather than being marginalized or scorned for his ethnic background as some have claimed, the young Sigmund Freud found in Vienna an intensely Jewish setting where he had every prospect of advancement through dint of hard work.

In 1881, Freud graduated in medicine, having taken eight years to complete the degree because of repeated distractions caused by his scientific curiosity. One of these distractions had been research on the nervous systems of lower forms of sea life, and Freud went on to do a residency in neurology,

in the course of which he spent five months on Meynert's service in the Vienna General Hospital. In 1885, just after receiving his title as docent, Freud traveled to Paris and was permitted to observe at Charcot's clinic through the winter of that year—a period when Charcot himself was just at the height of his obsession with "hysteria." Freud then returned to Vienna to set himself up in the private practice of neurology.[4]

As Freud opened his office, he was just one among many conventional neurologists, and he used the standard treatments of the day on the dysphoric, largely female population that was then the mainstay of private-practice neurology. Freud hypnotized his early patients, trying to cure the highly neurotic Elise Gomperz, wife of professor Theodor Gomperz, through suggestion. (Her husband complained that the hypnosis had made her worse.)[5] He also applied electrotherapy to his patients; in 1894, for example, he treated the supposedly "neuralgic" arm of Erwin Stransky's father with faradization (Stranksy's father had cancer).[6] Because business was so poor, at one point Freud contemplated hiring on at a water-cure clinic.[7] For many years, Freud maintained a run-of-the-mill neurology practice.

But one aspect of, or misadventure in, his professional life was not conventional. Just after returning from Paris in 1886, he was asked by his former chief Meynert to give a lecture on hysteria to the Society of Physicians. In front of an audience who considered themselves the elite physicians of the Western world, Freud waxed fully about the wonders of Charcot's approach. "This went down poorly with the Vienna greats," psychiatry professor Julius Wagner-Jauregg recalled many years later. "[Heinrich] Bamberger and Meynert gave Freud a big rap on the knuckles in the discussion, and Freud fell almost into disgrace within the faculty." Then, continued Wagner, a distinguished Viennese family doctor with an extensive consulting practice in the Jewish community named Josef Breuer took pity on Freud "as a neurologist without patients." "Breuer now came up with work for Freud in referring hysterical Jewish girls to him for treatment."[8] In 1895, Freud and Breuer published a book together, *Studies on Hysteria*, containing Breuer's history of a young woman he named "Anna O," as well as Freud's account of several other cases.[9] (Breuer's approach to the case became christened "cathartic therapy.")

Freud was struck by what he believed to be a sexual element in the stories of these young women. There was, for example, Elisabeth von R, who later remembered, after finding her sister dead, that she had previously longed for

her sister's husband. She thought guiltily to herself, "Now he is free again and I can be his wife." Freud believed that the patient developed a hysterical paralysis as a result of dealing with this conflict.[10] These experiences could be generalized, Freud thought. He theorized that much hysteria and anxiety could be explained on the grounds of patients' early experiences with sexual trauma and with adult experiences of sexual abstinence, masturbation, and such practices as coitus interruptus. After 1897, Freud came to believe that it was not actual sexual trauma but fantasies of incest in childhood that opened the wellsprings of neurosis in his adult female patients.[11]

Thus in his practice, Freud began talking to his bewildered patients ever more about sex. As he wrote to his friend Wilhelm Fliess in 1893, a Berlin family doctor whom Freud had chosen as a special confidant, "The sexual business attracts people who are all stunned and then go away won over after having exclaimed, 'No one has ever asked me about that before!'" [12] But in eliciting all these sexual memories—in a population of young middle-class women with presumably normal hormonal drives who were cloistered in conservative Jewish families—Freud pressed his patients very hard, to the point of suggesting them into recalling events that may not have occurred or of vastly exalting the importance of trivia. By applying pressure to one patient's forehead, he was able to get her to bring an aria from Carmen into association with a longing for sexual caresses. The patient fled the interview.[13]

This urgent dredging for sexual material became characteristic of the early days of psychoanalysis. Budapest analyst Sandor Ferenczi would plunge immediately into sexual matters in the clinical interview. He was once called to the bedside of a young woman who had become delirious after breaking a leg on a bobsled run. When the patient proved resistant to the sexual line of interrogation, Ferenczi began quizzing the patient's mother, who reported that the patient had once fainted while out on a drive. Ferenczi interpreted this as evidence of the patient's desire for sex with the coachman.[14]

The early analysts became well known for searching out sexual material. Viennese psychiatrist Emil Raimann, who knew Freud and his patients well, complained that Freud was able to persuade these complaisant and easily suggestible young women to say anything he wished them to. "The patients who consult Freud know in advance the information he wants to extract from them. These are patients who have let themselves be convinced of the causal significance of their sexual memories. Individuals in whom sexual motives

play no role are aware that they would consult Freud in vain." (Raimann noted that in working-class families in Vienna there was plenty of sexual contact, even incest, but no hysteria. Yet among the closely guarded young women of the city's better families, where there was no possibility of sexual trauma, hysteria flourished.)[15] In the late 1890s, Raimann had worked summers at a private nervous clinic at Purkersdorf near Vienna, and was quite familiar with the middle-class families from whom Freud recruited his patients: "Once he [Freud] had called their attention to sex, this would automatically replace any other variety of pathogenic memory." Among these patients, said Raimann, "little remained in the way of daily concerns save sexual matters, resulting in an ennui and boredom that the patients tried to drown out by reading the latest and most arousing novels.... In these circles Freud quickly became well-known and valued as a sex researcher."[16]

Yet the culture of the European middle classes at the *fin de siècle* was receptive to sex. Although the young Freudians did not claim to monopolize the subject, they were nonetheless the only ones to offer a road map of how one got from sexual desire and repression of it to neurosis. On the basis of this map, psychoanalysis, a term Freud first used in 1896, would turn into a movements.[17] It launched itself on the world as a group of doctrines comprising three main areas: study of the patient's resistance to thoughts that attempted to press into the conscious mind from the unconscious; concentration on the causal significance of sexual matters; and an emphasis on the centrality of early childhood experiences.[18] The core doctrine, from which Freud never wavered, was that neurotic symptoms represented a trade-off between sexual and aggressive drives and the requirements of reality.

In 1902, Freud set up a discussion group that met on Wednesday evenings in his home. This was his first attempt to go beyond communicating with his (by now ex-) friend Fliess and to recruit a group of followers. The schisms and rivalries that plagued this Wednesday group in its brief existence before World War I illuminated the basic problems that dogged psychoanalysis throughout: Freud was so intent on propagating his own views that, by turning psychoanalysis into a movement rather than a method of studying subrational psychology, he denied analysis the possibility of ever acquiring a scientific footing.

The master's insights were to become articles of faith, incapable of disproof. And the efforts of others to criticize Freud's wisdom would always be considered evidence of "resistance," of personal pathology, never as

scientific hypotheses to be dealt with in the way that science treats all hypotheses. Alfred Adler fell away, as did Wilhelm Stekel, Freud's physician-patient who had suggested establishing the Wednesday group in the first place. Such far-distant fans of analysis as the Zurich academics Carl Jung and Eugen Bleuler would soon turn heretic, as did later many others. The efforts of all these critical individuals to nudge Freud away from the bedrock of childhood sexuality on which he built his theories would fail. But a core of faithful remained. And it was these loyal captains who, in the belief that they possessed an inner truth, took psychoanalysis to the wide world.

The Berlin analyst Franz Alexander, who in the early 1930s established a beachhead for psychosomatic medicine in Chicago, explained how it felt to be "a member of such a courageous pioneering group." "In the main, you were right and the *world* was wrong. Even at the first rough approach to your subject, there was sufficient evidence for your teachings. You knew positively that ... repressed sexual impulses were the main sources of neurosis of the Victorian and post-Victorian Westerner, and, above all, that sexuality was there from the beginning of life, and its objects in the infant were incestuous."[19]

Did Freud and his followers really know these truths? Or were they simply self-suggesting one another into accepting highly dubious propositions as being somehow "confirmed"? Freud tended to see himself more as an adventurer than a scientist, once telling Fliess flatly, "I am actually not at all a man of science, not an observer, not an experimenter, not a thinker. I am by temperament nothing but a conquistador-an adventurer, if you want it translated—with all the curiosity, daring, and tenacity characteristic of a man of this sort."[20] His inner circle was rife with toadyism, for the other analysts were economically dependent on Freud for referrals. (He kept a pile of their calling cards in his drawer, and would dole them out to patients according to his whim.)[21] "Freud never realized how much of a suggestive impact he had on his followers," writes historian Paul Roazen, "and therefore could be led to think that his findings were being genuinely confirmed by independent observers."[22] The issue of validity would therefore haunt psychoanalysis until its eclipse within psychiatry.

A second troubling issue was the extent to which psychoanalysis represented a specific therapy as opposed to a worldview. It offered indeed a comprehensive picture of human connectedness within society, able to account for the shapes of water faucets (penislike) as easily as explaining the

fear of intimacy (suppressed homosexual longings). But Freud himself was uneasy about whether psychoanalysis could actually make people better. He published few case histories and seems to have felt most comfortable with analysis at the societal level, of which the best known example is his book *Civilization and Its Discontents*, written in 1930.[23] After a colleague told him about a therapeutic success, Freud is reported to have looked up almost astonished and said, "Oh yes, you can also cure people with analysis."[24] On another occasion, Freud confided to Ludwig Binswanger, the superintendent of a luxurious private nervous clinic in Kreuzlingen, Switzerland, "I often console myself with the idea, that even though we achieve so little therapeutically, at least we understand why more cannot be achieved. In this sense our therapy seems to me to be the only rational one."[25] As psychoanalysis set out to take over psychiatry, therefore, it was with a doctrine that was therapeutically uncertain, intellectually highly speculative to say the least, and best adapted to the psychological needs of a deracinated group in transition: young middle-class Jewish women who aspired to be like their non-Jewish counterparts. It would be hard to imagine a therapy less appropriate for the needs of people with serious psychiatric illnesses.

Given the intrinsic inappropriateness of psychoanalysis for psychiatry, there must have been some other force driving it forward in Europe than the power of the idea itself. That force was middle-class enthusiasm. Freud's ideas proved tremendously popular among the educated classes as a codification of the kind of search for self-knowledge that had run through bourgeois culture throughout the entire second half of the century. Psychoanalysis was to therapy as expressionism was to art: Both represented exquisite versions of the search for insight. Psychoanalytic ideas were sufficiently *à la page* in prewar Berlin that the readers of Grete Meisel-Hess's novel *The Intellectuals*, published in 1911, would have resonated to "Erika's" psychiatric adventures:

> "I am sick," she decided. "I must go to the doctor." As she had long been curious about "this psycho-analytic" procedure, she sought out a "famous psychiatrist."
>
> After hearing her tale, he told her, "You have repressed your painful sexual experiences instead of overpowering them.... Nicht wahr?"
>
> Erika nodded. He continued, "It's important to open your eyes and to recall into your consciousness that repressed experience in its true form."

> He used the verb "abreact" and explained to her basic notions about dreams and her erogenous zones. He gave her a gynecologic exam, apparently on the theory that a tilted uterus might be causing her hysteria.
>
> "Since everything's in order down there," he said, "I will need to treat you only psycho-analytically." He told her that her "hysterical affective psychosis" was curable and that he would hypnotize her.
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> The consultation came to an end after he awakened her from hypnotic sleep by touching her eyelids. Erika was well again.[26]

We recognize this as a travesty of psychoanalysis in the form that Freud's doctrine was later codified—Freud stopped hypnotizing patients late in the 1890s—and psychoanalysis did not incorporate hypnosis. Yet this is how psychoanalysis was understood before the analysts founded their own training institutes to standardize the technique. Berlin's psychoanalytic society, the first in Central Europe, was founded in 1908 (after 1920 actual training was conducted in a psychoanalytic outpatient clinic).[27] By 1925, psychoanalysis had become so fashionable among the Berlin middle classes that people would chat about their "Minko's," short for "Minderwertigkeitskomplex," or inferiority complex.[28]

In Vienna, too, the "psychoanalytic infestation" (as novelist Elias Canetti put it) had taken firm hold. "At that time you couldn't say anything in conversation without having it nullified with some kind of facile reference to unconscious motivation. The unspeakable boredom that these people gave off, the sterility that resulted from this, was of apparent concern to few."[29] When Canetti himself had some kind of absence fit, the kindly old family "Dr. Laub" was summoned. He reassured Canetti's mother: Just let him be. "That's good for the Oedipus."[30]

Regular psychiatrists were bemused at the grassfire spread of psychoanalysis within the middle classes. One physician at the Budapest psychiatric clinic tried to account for it along the following lines: "The flood of patients seeking salvation through psychoanalysis is explainable partly from the publicity, partly from the receptiveness of our time to introversion and introspection." It was a procedure of obvious appeal to "hypersexual neurotics," he said.[31] Thus we have a core of physicians dubious, even contemptuous of "hypersexual neurotics" and their problems, and an educated middle class keening at the doctor's office for further self-insight. The stage was set for the virtual destruction of traditional, asylum-based

psychiatry.

The Battle Begins

How psychoanalysis grew into a movement is a story in its own right. What interests us here is its efforts to take over psychiatry. Between the late 1890s, when Freud's new notions of therapy started to interest outsiders, and the 1960s, the high-water mark of the psychoanalytic movement, Freudian doctrine made deep inroads into psychiatry. These analytic incursions provoked a tremendous struggle within psychiatry, a discipline previously oriented to biology and not to psychology. If in the end analysis won, it was not necessarily because of the power of Freud's ideas but because analysis opened the road to private practice.

What most physicians understood initially by psychoanalysis was not its classic form, meaning 50-minute hours five days a week, a silent psychiatrist seated behind a patient on a couch who was reciting dream memories or doing word associations. That form tended to be disseminated by the psychoanalytic training institutes that flourished in the big cities after the 1930s. The early form of analysis, disseminated by Freud's own writings, emphasized interrogation of the patient about his or her erotic life in the context of a close doctor-patient relationship. Freud himself was actually quite "unorthodox," often going to his patients' homes, socializing with them, and indulging in behavior that would later have been considered unconventional.[32] Yet the early Freud did not offer some anodyne version of psychotherapy. Rather it was a thorough grilling on such matters as coitus interruptus, masturbation, and early memories of sex and desire.

As psychiatry resisted the psychoanalytic incursion, two main themes came to the fore: skepticism about Freud's views on the sexual causes of mental illness, and the professoriate's reluctance to see psychiatry diverted from psychosis to neurosis.

The battle was joined first in Central Europe. Among those who opposed Freud, the dominant note was simple disbelief in Freud's sexual reductionism. It is inconceivable, said Gustav Aschaffenburg, a former assistant of Kraepelin and, in 1906, psychiatry professor in Cologne, that masturbation causes "the repression of affect." Nor did Aschaffenburg find it

likely that sexual abstinence was the main cause of anxiety. Freud must be getting his results by putting words in the patients' mouths, Aschaffenburg said, and concluded that the whole edifice of psychoanalytic theory was a triumph of suggestion.[33] Adolf Friedlander, chief physician at a private clinic near Frankfurt, was judged in the psychoanalytic movement to be a notorious enemy of Freud. Yet Friedlander had sinned mainly by skewering various figures within psychoanalysis for their negative statements about women.[34] His position was actually quite moderate: "Psychoanalysis [meaning psychotherapy] is in and of itself indispensable for neurologists and psychiatrists. But *sexual* psychoanalysis strikes many of us as doubtful or unnecessary."[35] These comments are typical of a whole range of reaction in Germany, a disbelief that stemmed not necessarily from prudishness, for many critics of psychoanalysis had nothing against other forms of psychotherapy in which intimate details might also emerge, but from sheer unwillingness to accept that neurotic pathology boiled down to sex.

The professors represented a slightly different source of hostility. The analysts claimed quite correctly that most of the chair-holders in psychiatry were lined up against them, with the well-known exception of Eugen Bleuler in Zurich and the less-well-known exception of Karl Bonhoeffer in Berlin. (In 1914, for example, Bonhoeffer asked candidates for medical-officer-of-health posts to write an exam question on "The significance of psychoanalysis for psychiatry."[36]) To be sure, the professors, like most other observers, disliked the movement's sexual reductionism. Thus Germany's leading neuropsychiatrist, Adolf Strumpell, called the Viennese obsessed by sex.[37] "I find this Freudian cult nauseating," said Zurich psychiatry professor August Forel in 1907, and later told of patients whom psychoanalysis had worsened by fixating them upon sexual matters.[38]

Yet sex was not the sticking point for most of the professoriate. And if Otto Binswanger, Oswald Bumke, Alfred Hoche, Emil Kraepelin, Konrad Rieger, and others saw fit to attack psychoanalysis, there must have been some other circumstance. These men without exception had backgrounds in asylum psychiatry. For them mental illness meant psychosis, a disorder requiring institutional care. Seeing the younger psychiatrists abandon the psychoses and chase after a doctrine that promised relief mainly for neurotic illness was probably more than the senior professors could stand. It represented a renunciation of their life's work and of their sense of the very mission of psychiatry. For they quite correctly perceived Freud and the

analysts to be redirecting the discipline toward the understanding and therapy of the psychoneuroses—the psychiatric disorders of daily life seen outside the asylum.

There was almost a professorial bewilderment at seeing psychiatry's entire center of gravity shift.[39] Würzburg professor Konrad Rieger captured this regret perfectly in 1896: "I cannot imagine that an experienced alienist would be able to read [Freud] without sensing true disgust. And the reason for this disgust would be that the author assigns great importance to paranoid chatter of a sexual nature ... even when it has not been wholly invented. This kind of effort can only lead to a ghastly 'village-crone psychiatry *[Altweiber-Psychiatrie].*'"[40] The problem with village-crone psychiatry was that it addressed people who were unhappy but not mad.

Rieger and company did well to be alarmed, for in the years before World War I, younger psychiatrists were indeed flocking to psychoanalysis as a form of psychotherapy. There are two themes here: first, the willingness to use some form of analysis as a way of prying open the doctor-patient relationship, of introducing psychological sensitivity to psychiatry without necessarily becoming a "psychoanalyst" oneself; second, the embracing of Freud's doctrine willy-nilly because it appealed to patients and had started to become celebrated in the patients' world. As internist and psychiatrist Viktor von Weizsäcker said of his youthful encounters with psychoanalysis, "Medicine as it was practiced in university clinics then [around 1914] awakened in me and in many other young physicians the greatest doubt. Rounds were becoming increasingly impersonal; the wards, the dominance of laboratory tests and [electrocardiogram] curves revolted me. There was never a one-on-one conversation between doctor and patient.... What a contrast with what the psychotherapists were doing, who had spent their entire lives just having intimate dialogues in their practices!"[41] For Weizsäcker, it was important not to become a formal psychoanalyst but to use this new perspective as a way of talking with patients. He belonged to a whole cohort of young physicians who saw in analysis not a holy shrine but a way of communicating.[42]

Another group of psychiatrists, perhaps less psychologically minded but more profit oriented, embraced psychoanalysis because they wanted to attract patients. These were the owners of the private nervous clinics. And their fondness for what they advertised as "psychoanalysis" was the supply-side response to the rising demand for "Freudianism" among the middle classes in

the years around World War I. For these were the classes that became patients in such clinics. The highly competitive private clinics would generally offer whatever therapies seemed trendy, diet therapy in the 1890s or "sun and air" cures in the belle époque.[43] After around 1910, it was psychoanalysis that was in fashion.

Freud's and Breuer's version of psychotherapy as enunciated in 1895 was called cathartic therapy. It was picked up immediately in the world of the private clinics. As early as 1900, Wolfgang Warda, owner of a private nervous clinic in Blankenburg, a spa in Thuringia, was telling colleagues of his success with the "cathartic method of Breuer and Freud."[44] After Freud abandoned the notion of catharsis as curative and went over to insight-oriented therapy, his supporters in the private sector began to multiply. After visiting Freud in Vienna in 1907, young Ludwig Binswanger at the Bellevue Clinic in Kreuzlingen became an adept. After the death of Binswanger's father Robert in 1910, Ludwig took over the clinic and began advertising to doctors and patients his enthusiasm for the new therapy. "In those days," said Binswanger, "I was still convinced that every psychoneurosis, and many psychoses and psychopathic personalities, could be healed or at least improved with psychoanalysis."[45] Despite demands by mainline psychiatrists to boycott such clinics, Bellevue flourished with the new approach.[46] There were several other prewar examples like Binswanger, such as Otto Juliusburger, a founding member of the Berlin psychoanalytic society. On staff at a large private asylum in the Berlin suburb of Lankwitz, Juliusburger was very sensitive to the accusation that psychoanalysis and Jewishness somehow ran together.[47]

World War I made a big difference in public acceptance of psychoanalysis, perhaps because Freud's views of the death instinct and aggression seemed to illuminate the war's awesome irrationality. (In 1920 Freud argued that two fundamental instincts existed—the life and death instincts, or Eros and Thanatos, rather than just the sexual instinct as he previously believed.) After the war, it became the rage to include psychoanalysis in one's therapeutic palette. Many clinics that previously would have nothing to do with psychotherapy were now proclaiming their attachment to psychoanalysis. By 1927, for example, Wilhelm Rohrbach was insisting that he offered "psychoanalysis" in his establishment in the Kassel suburb of Wilhelmshöhe among a long list of therapies that otherwise included "all varieties of baths, modern light and air treatments, electrotherapy in the newest forms, terrain

therapy, gymnastics and massage ... and hypnotic and nonhypnotic suggestive treatments." [48] (Rohrbach had taken the clinic over just after the war from a former owner who had disgraced himself by assaulting the patients.) With the Nazi seizure of power psychoanalysis vanished from private clinics of Germany.

A final domain of psychiatry into which psychoanalysis pushed was the public asylum itself. Here it was mainly keen young physicians, avid to try anything new, who picked up Freud's doctrine. A typical early instance: In 1903, a junior physician, Hans Eglauer, at the Lower Austrian asylum of Kierling-Gugging outside of Vienna, noted in a patient's chart that he had attempted to investigate her "inner life" (*Seelenleben)*, but had little success because she appeared to him too unintelligent and uneducated. [49] Eglauer was almost certainly influenced in this enterprise by Freud's teachings, which by 1903 had become celebrated in Vienna. Novelist and psychiatrist Alfred Doblin, who before the Nazi takeover worked in the asylum of Berlin-Buch, gave in his 1929 novel Berlin *Alexanderplatz* what may have been an autobiographical account (from a prewar experience) of two young psychiatrists trying to use psychoanalysis on a psychotic patient, the mythical "Franz Biberkopf."[50] The novel reads:

> "[The young doctors] are inclined to consider Franz Biberkopf's trouble as psychogenic, that is, his rigidity comes from the soul, it is a pathological condition of inhibitions and constraints which would be cleared up by an analysis (perhaps it emanates from early psychic states)." Yet they cannot get Biberkopf to talk. The analysis fails.
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> The young doctors then talk the case over with the weary old chief physician of the asylum. He laughs at their efforts: "When a confirmed jail-bird like him sees two young gentlemen who talk a lot of rubbish about him ... and want to do some prayer-healing with him, well, take it from me, you're simply soft mash for a chap like that."
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> The young doctors protest, "But he's inhibited, sir, in our view it is a repression, conditioned by a psychic crisis, a loss of contact with reality, due to disappointment, failure, perhaps infantile and instinctive demands on reality...."
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> "Psychic crisis be damned," says the old physician. "You're really a master-healer, three cheers for the new therapy, and you can send a telegram of congratulation to Freud in Vienna."[51]

Elsewhere, enthusiastic young efforts to bring psychoanalysis to the asylum also met with similar rebuffs. Around 1907, Arthur Muthmann attempted to import analysis to Basel's Friedmatt asylum, but was rejected by the chief physician, who apparently thought so little of Muthmann's judgment that he refused to sponsor Muthmann's postdoctoral habilitation research.[52] Another junior Swiss doctor, Karl Gehry, on staff at the Rheinau asylum near Schaffhausen, was so enthusiastic about Forel's work on the psychic life of ants that he decided to read Freud's *The Psychopathology of Everyday Life*, published as a book in 1904. Encouraged by his colleague Franz Riklin, a well-known advocate of analysis, Gehry began attending Jung's and Bleuler's psychoanalytically oriented seminar at the Burghölzli, the university psychiatric clinic in Zurich. The contagion seems to have spread to the entire Rheinau staff except for the medical director who, Gehry said, "had on blinkers when it came to sex." Gehry abandoned Freud, however, when all of his efforts to analyze these very ill patients failed.[53]

Thus in Central Europe the efforts of psychoanalysis to press into psychiatry ran along three lines: attempts to make the doctor-patient relationship psychologically more sensitive, to improve one's competitive position in the private sector, and to achieve some glimmer of therapeutic hope in the public sector. All three would help lay the basis for office practice, as physicians who had trained in these various settings understood how much wider the sweep of psychiatry became once the discipline absorbed a psychological approach to neurotic illness.

By the first annual meeting in 1926 in Baden-Baden of the General Medical Congress on Psychotherapy, an organization whose inspiration was heavily psychoanalytic, psychotherapy in Central Europe had come to mean applying in private practice some kind of technique derived from psychoanalysis. Of the nearly 500 physicians attending that congress, 70 percent were in private practice. And while a few pediatricians and dermatologists did take part, the majority of doctors present were psychiatrists.[54] Thus by the mid-1920s in Central Europe, the spirit of psychoanalysis had made great inroads into psychiatry.

The main points of the general story are evident from this account of events in Central Europe, which it is unnecessary to extend to all other countries. After World War I, young psychiatrists everywhere who were interested in office practice based on psychotherapy started thinking psychoanalytically, though not necessarily seeking to become full-fledged

analysts themselves. By the end of the 1920s, what they were reading about psychotherapy and the like was heavily analytic in nature. [55] This was a trend in which every country shared and illustrates the extent to which psychiatry was coming to mean some kind of therapy based on the doctor-patient relationship as opposed to dumping patients into baths in an institution.

By the 1920s, psychiatry was blossoming outside the asylum. The intellectual trend giving energy to this blooming was psychoanalysis. Until this point, the center of gravity of the story had been Europe, for it was mainly in Central Europe that the beat to the international stride was drummed. Yet in the 1930s, the history of psychiatry underwent a momentous change. The rise of Nazism in Germany and Austria snuffed out literally from one moment to the next the rich scientific discipline of psychiatry that had flourished on that soil for a century and a half. Many of its distinguished Jewish practitioners were murdered in the Holocaust or dispersed their energies in the chaos of emigration. By 1945, psychiatry as a discipline was dead in Germany and Austria.

After the 1930s, the world epicenter of psychiatry shifted to the United States. It was in America that psychoanalysis flourished beyond the wildest dreams of its Viennese founders. In the United States, psychoanalysis took over the profession of psychiatry for a period of about three decades. Only in the 1970s would American psychiatry begin to recover from its analytic infatuation. And because postwar America replaced Central Europe as psychiatry's driving force, what happened in the United States had a capital importance for the evolution of the discipline in general.

American Origins

Psychoanalysis had great significance for the history of psychiatry in the United States. Under the influence of Freud's teachings, American psychiatry accomplished the switch from psychosis to neurosis as the object of study, and from the asylum to Main Street as the venue for practice. The price of this advance was that psychoanalysis infiltrated American psychiatry far more deeply than elsewhere, causing scientific stagnation and increasing disengagement from the rest of medicine.

Psychoanalysis was important in anchoring American psychiatrists in the

office. Yet the trend from asylum to private practice began before that period and for independent reasons. As early as the 1880s, psychiatry started to establish a freehold in the form of outpatient clinics, some based in asylums and some not. In November 1885, Philadelphia's John Chapin, superintendent of the Pennsylvania Hospital for the Insane, opened a clinic in the outpatient department of the Pennsylvania General Hospital, the parent organization of the asylum. The clinic saw about a hundred patients a year, half of them psychiatric, half neurological.[56] This was the first significant American facility for treating noninstitutionalized psychiatric patients. Twelve years later, in 1897, Walter Channing established a "mental clinic" at the Boston Dispensary to enable giving clinical instruction to medical students and young doctors. Channing was the owner of a private nervous clinic in the suburb of Brookline and taught psychiatry at various institutions in the area. "There is a big field for the psychiatrist outside of the hospital," he said.[57]

Indeed. Around the turn of the century, psychiatry began to care for patients in the community on a number of fronts. In 1906, for example, in New York State a charitable association initiated a program of after-care for discharged patients under the guidance of Louise L. Schuyler. Before World War I, several psychiatric hospitals opened their own outpatient clinics, the Henry Phipps Psychiatric Clinic in Baltimore under Adolf Meyer being the first university hospital to do so.[58]

Psychiatry further reached out with the founding in 1909 of the National Committee for Mental Hygiene. A book by ex-psychiatric patient Clifford Beers, *A Mind That Found Itself* (published in 1908), prompted a number of prominent figures such as Meyer and William James to promote the concept of "mental hygiene." In subsequent years, the mental-hygiene movement involved psychiatrists in numerous plans to improve the "mental health" of Americans through various well-meaning efforts.[59]

Thus even before psychoanalysis, private-practice psychiatry, dedicated to the lesser but extremely common psychiatric illnesses, was acquiring a Main Street beachhead. By 1927, George Kline, commissioner of the Massachusetts department of mental health and then president of the American Psychiatric Association, was able to write, "The past decade has witnessed a remarkable extension of the sphere of psychiatry beyond the walls of the mental hospital."[60] Of psychiatrists attending the annual meeting of the American Psychiatric Association in 1910, only 3.2 percent were in

private practice. By 1921, the figure had risen to 7.3 percent.[61] Yet it was psychoanalysis that permitted this beachhead to be expanded into a victorious onmarch that would ultimately sweep almost the entire discipline. Psychoanalysis was the caisson on which American psychiatry rode triumphantly into private practice.

From the very beginning, Freud's writings were received enthusiastically in America. Well before the organization of local psychoanalytic societies, individual physicians who themselves often spoke and read German as a result of postgraduate training in Berlin or Vienna were picking up Freud's works and trying to apply them. Around Boston, for example, Freud first started to be read around 1894—a year before the publication *of Studies on Hysteria*. Harvard psychologist William James went on to kindle interest among a small number of physicians who formed a discussion group meeting in the home of Morton Prince, a professor of neurology at Tufts University. (Prince, who founded the *Journal of Abnormal and Social Psychology* in 1906, became well known among American physicians and psychologists for importing a psychological, semi-Freudian perspective into the study of neurosis.) James Jackson Putnam, professor of neurology at Harvard, contributed a paper to the first issue of this journal on his experiences with what he called "psychoanalysis" at the Massachusetts General Hospital. This represented the official launch of Freud's doctrines in America though that had not really been Putnam's intention; he was interested in experimenting broadly with psychotherapy as opposed to psychoanalysis in particular.[62]

In New York large numbers of neurologists and family doctors acquainted themselves with Freud's techniques. In 1909, Abraham Brill, an Austrian-born neurologist who had emigrated to the United States at the age of 15, translated Freud's *Studies on Hysteria* into a rather clumsy English. Brill had traveled back to Austria and Switzerland, knew Freud (he considered his chats with the Professor to be a training analysis), and claimed credit as the actual founder of psychoanalysis in the United States.[63] Analysis flourished particularly among New York's society neurologists who, as in Europe, had been the principal previous practitioners of psychotherapy. By 1922, there were said to be more than 500 "unofficial" analysts in the city.[64]

In September 1909, accompanied by Jung and Ferenczi, Freud came to the United States to lecture at the invitation of Stanley Hall of Clark University. The tour gave a great boost to psychoanalysis among American physicians and the public. Freud however hated the United States, despising many of his

American enthusiasts. (He considered Prince “an arrogant ass who would be conspicuous even in our menagerie.”) Yet the press interviews that he gave aroused great curiosity. In the immediate wake of the visit, there was a spurt of medical praise of analysis, and psychoanalysis became launched as a movement in the United States.[65]

The building of the psychoanalytic movement was accomplished in two steps. First came the founding of local psychoanalytic societies, the professional organizations of the analysts themselves. Second came the psychoanalytic training institutes, usually but not always under the control of the local societies. These served to educate future generations of analysts in the intricacies of psychoanalytic theory and technique, so that a commonly understood body of doctrine could be handed down from one generation to the next. As in Europe, the societies and training institutes were separate from each other. Unlike Europe, however, in the United States only physicians were eligible to apply for training. Although there was no good reason why nonphysicians could not do analysis just as effectively—there being nothing intrinsically medical about plumbing the unconscious mind—American medical psychoanalysts had a horror of lay competition. And from their viewpoint, rightly so. Given that psychoanalysis had been psychiatry’s ticket out of the asylum, the last thing the American analysts wanted was to break their monopoly by sharing it with psychologists and social workers avid to have a go at analysis themselves.[66]

The first local psychoanalytic society in the United States was founded by Brill in New York in February 1911. At the outset, the theme of escaping the asylum was significant. Of the 15 founding members, 10 were associated with the Manhattan State Hospital.[67] Another member, Bronislaw Onuf, was chief physician at Knickerbocker Hall in Amityville, Long Island, a private nervous clinic that would soon start offering psychoanalysis, if in fact Onuf was not already applying it in 1911.[68] “These matters were eagerly discussed at our staff meetings, and at the Ward’s Island Psychiatric Society,” said David Henderson, an Edinburgh physician who between 1908 and 1911 was doing a psychiatry residency on Ward’s Island. At one meeting of the society, Adolf Meyer gave a paper entitled “A Discussion of Some Fundamental Issues in Freud’s Psycho-Analysis.”[69] So there is no doubt that asylum psychiatry was heavily implicated at the very birth of American psychoanalysis.

In May 1911, Ernest Jones came down to Baltimore from Toronto, where

he was then in exile from England, and arranged for the founding meeting of the American Psychoanalytic Association. Here Jones and associates were building on another organization, the American Psychopathological Association, which Putnam, Meyer, August Hoch, Jones, and others had founded in Washington the previous year. In 1911, the new psychoanalytic society would meet at the same time and in the same Baltimore hotel as the psychopathological society.[70] The American Psychoanalytic Association was not to be a local society but was open to anyone in the country who declared an interest in Freud's ideas.[71] Until 1932, this nationwide organization amounted to little, controlled as it was by New Yorkers who additionally had their own robust local society. It was peopled by the somewhat quirky and reflective physicians who in those days tended to express an interest in Freudian ideas.

Other local societies were soon established elsewhere. Washington, DC, acquired its initial psychoanalytic society in 1914 (which flickered in and out of life until being solidly refounded in 1930). Boston's first psychoanalytic society was founded in 1930, Chicago's in 1931, numbering among its members such European heavyweights as Franz Alexander and Therese Benedek.[72] In 1932, the American Psychoanalytic Association reorganized itself as a federation of local societies, and from that point on, national standards became applied to local organization and training. The training institutes founded in the 1930s applied these standards with a vengeance. Soon an orthodox template for conducting psychoanalysis began to press individual idiosyncracies about hypnotism, cathartic therapy, and the many other practices once identified as "psychoanalysis" into a uniform pattern that would be recognizable from coast to coast.

From the viewpoint of the history of psychiatry, the most interesting aspect of the history of psychoanalysis is its efforts to take over psychiatry. How does a small discipline set out to engulf a big one? Overlapping membership is one tactic. Ideally, all the analysts would end up as psychiatrists, and vice versa to a lesser extent. Starting in the 1920s, big psychoanalytic names showed up increasingly at psychiatric meetings. At the 1928 psychiatric meeting in Minneapolis, for example, present among the 32 psychiatrists in private practice were such figures as Leo Bartemeier, who years later would help found the Detroit psychoanalytic group, or Clarence Oberndorf, a founding member of the New York Psychoanalytic Society.[73]

More and more analysts were becoming psychiatrists. They had no choice.

In 1938, the American Psychoanalytic Association required candidates for a training analysis to have completed at least one year of a psychiatric residency. During the 1940s, several local institutes raised this requirement to two years, and the association began encouraging psychiatry residents to commence simultaneously a training program at a nearby psychoanalytic institute (if one were in fact nearby). By 1944, 70 percent of American psychoanalysts had qualified in psychiatry (sometimes in neurology as well: a common examination board for psychiatry and neurology was established in 1934). By 1953, 82 percent of the members of the American Psychoanalytic Association were simultaneously members of the American Psychiatric Association. The psychoanalytic association had by now stipulated that would-be analysts must complete at least three years of a psychiatric residency before commencing psychoanalytic training.[74] The analysts had thus joined psychiatry.

The psychiatrists and psychoanalysts moved closer together in organizational terms as well. From 1924 on, the American Psychiatric Association and the American Psychoanalytic Association would hold their meetings at the same time in the same city. Nine years later, in 1933, the American Psychiatric Association started a special section of psychoanalysis for interested members.[75] Although there initially had been a huge fight on the APA's executive board about embracing analysis, the psychiatric organization could see which way the wind was blowing. It was blowing toward private practice, and that meant encouraging psychotherapy.

In the late 1930s, the analysts themselves were pushing hard for their movement's breakout into general psychiatry. Lawrence Kubie, president of the New York Psychoanalytic Society, urged that the psychoanalytic education of psychiatrists be carried on in units attached to mental hospitals and medical schools. "In this way training in psychoanalysis could gradually become part of the training of the house-physician in every mental hospital," he argued.[76] As Franz Alexander of the Chicago institute declared in a 1939 letter to Ernest Jones, "[Psychoanalysis] is rapidly becoming part of general medical practice and training."[77]

By the entry of the United States into World War II, psychoanalysis had captured the basics of training in psychiatry at the undergraduate and graduate level. In 1942, a major study of psychiatric education concluded that, while one could not turn all medical students into analysts, "one can have no quarrel with the introduction of at least the basic psychoanalytical

concepts in the course of psychopathology [for the medical students], and indeed it is difficult to see how psychopathology can be taught without these concepts."[78] The psychiatrists who went marching off to cure combat fatigue during the war were armed with psychoanalytic doctrine.

With analysis skyrocketing in psychiatry, the public was not long in latching onto this appealing new therapy, at least those who could afford it. Just as the middle classes of Berlin had chattered about their "Minko's," middle-class Americans were breathless about their unconscious "defenses" that required dismantling. In Sally Pierce's 1929 memoir of her encounter with nervous disease, she describes having tried first one exclusive private nervous clinic then another. The rest cure, the being-yelled-at cure, the Dubois-style cure of rational persuasion: All had failed her. Then she stumbled into the hands of "Frank Gaylord," the psychoanalyst. "No permanent cure of a neurosis could take place," Dr. Gaylord told her, "until each unconscious cause of each neurotic symptom had been discovered and brought up into the patient's consciousness; not only brought up into consciousness, but discussed, examined, appraised, and finally understood and recognized by the patient for what it was: infantile, regressive stuff that blocked a progressive, adult living of life." Dr. Gaylord told her she would need daily appointments of an hour each time, stretching over many months. Finally after her "unconscious defenses" against the intrusion of reason had been broken down, she would be well again. Indeed after months of analysis, she did become well again. Here was apparent evidence that psychoanalysis worked. The American public that feasted on this and similar accounts increasingly put behind them the world of the private clinic with its amazonian nurses enforcing rest cures and embraced the Park Avenue practitioners of psychoanalysis.[79]

In 1935, *Fortune* magazine in a piece entitled "The 'Nervous Breakdown'" explained soberly that, "The suppression of the sexual instinct in childhood pushed certain experiences and desires deep into the unconscious, where they reappear in the adult as neuroses."[80] For the first time in history, the depressed businessman or the anxious housewife would seek out the services of a psychiatrist, and if the psychiatrist lived in New York, Boston, or Washington, chances were that he would be oriented to psychoanalysis.

The Arrival of the Europeans

History moves in odd ways. What ultimately converted a chic therapeutic boomlet into a mass ideology shaping almost every aspect of American thought and culture was the Holocaust. In the 1930s, fascism drove many analysts who were Jewish from Central Europe to the United States, where they lent the strippling little American movement the glamour and heft of the wide world. On the face of it, this massive transfer of culture from the German-speaking world to the English had positive results for psychoanalysis, reinforcing the homespun American heterodoxy with the prestige of internationally acclaimed figures.[81] In the long run, however, the migration of the European analysts proved fatal for psychoanalysis in the New World, for the refugees brought with them a stifling orthodoxy, a reflexive adherence to the views of Freud and his daughter Anna that American analysis was never able to outgrow and that ultimately caused, within medicine at least, its death from disbelief.

The number of refugee analysts from Europe was not large. Set against the 4,000 physicians from Germany and Austria who found shelter in the United States between 1933 and 1944, there were not more than 250 psychiatrists. And of these 250, not more than 50 were psychoanalysts.[82] Yet many were very well known. Consider who emigrated from the Berlin Psychoanalytic Society. Franz Alexander, whose name became synonymous with psychosomatic medicine, accepted a visiting professorship in psychiatry at the University of Chicago in 1930 (Alexander insisted that it be in "psychoanalysis"). Two years later, he founded a psychoanalytic training institute in Chicago. After the Nazi seizure of power, Alexander was no longer able to return. Sandor Rado of Berlin had initially been lured to New York in 1931 to organize a Berlin-style institute. Events would bar his return too. Otto Fenichel, who had written the first textbook of psychoanalysis, left Berlin in 1933, ending up in 1938 in Los Angeles where he worked as a training instructor.[83] When in May 1940, the Emergency Committee on Relief of the American Psychoanalytic Association inventoried refugee analysts with whom it recently had been in contact, there were eight names from Berlin on the list.[84]

Things were not easy for the new arrivals. Even though they read English, they often spoke the language poorly. When Fenichel, for example, visited at

the Menninger clinic in Topeka, he was asked to give a lecture. Aware that he often mispronounced words, he asked another emigré, Martin Grotjahn, for help. Grotjahn later recalled that Fenichel wanted to talk about something called "penis envoy." "That did not sound right," said Grotjahn, "and I tentatively suggested ... 'penis ivy.' A suggestion by another immigrant 'envy' was rejected as too unlikely by Otto and me." Fenichel's talk was "respected by all and understood by none. 'Penis envoy' finally brought down the house."[85]

Despite their linguistic deficiencies, the luminosity of the emigré analysts made them much in demand: Hermann Nunberg had left Vienna in 1932, initially for Philadelphia, then secured for himself a society practice on Park Avenue in New York; Freud's physician Felix Deutsch and his analyst wife Helene left Vienna in 1935 for a lovely home in Cambridge, Massachusetts. A swarm of analysts escaped Vienna in the great panic following the Nazi invasion of March 1938. Freud's lieutenant Paul Federn arrived in New York where he was, according to his friend psychiatrist Heinrich Meng, "at once counted among the leading psychiatrists of the country."[86] Heinz Hartmann ended up as a training analyst in New York. Lay analyst Ernst Kris became a professor at the New School for Social Research in New York (he had a PhD in art history, and had begun medical school, which he did not finish, in 1933). Beate ("Tola") Rank landed at the Judge Baker Guidance Center in Boston.[87]

One can only imagine the circumstances in which these men and women left Vienna, on a day's or hour's notice. Viennese dramatist Franz Werfel portrayed the scene, in which august and by-now middle-aged individuals, the Nazi tanks rumbling on the streets outside, would suddenly receive a phone call:

> *The doctor:* "You mean it's got to be tonight, over the border, no other choice? ... Otherwise ... What do you mean the worst can happen? ... That's terrific you'll take me in your car."
>
> The elderly physician begins to throw a few things together, then stops. He goes into his library and with hands shaking, starts taking down books. "What'll I take along?" He runs to his desk, "At least my medical diploma."

Here Werfel permits himself a bit of fantasy. The streetlights come on,

lighting up the bust of the doctor's old teacher. The physician hears in the room the teacher's voice: "Antisemitism is just a cyclical psychosis, from which manic-depressive nations suffer from time to time."

The doctor [addresses the bust]: "It's easy for you to make diagnoses, Herr Professor, because first of all you're not Jewish and second you're dead."[88]

That most of these very real emigré psychiatrists and psychoanalysts not only flourished but became the trend-setting force in the New World is a tribute to their energy and their bravery.

The emigré analysts had a major impact on American psychiatry and psychoanalysis. Paul Schilder, a member of the Vienna Psychoanalytic Society whose interests embraced the entire range of biological and dynamic psychiatry, left Vienna temporarily in 1928 for a post at Hopkins, from which he went in 1930 to a professorship at New York University and directorship of the clinical psychiatry division of Bellevue Psychiatric Hospital. Astonishingly, Schilder was rejected for membership by the New York Psychoanalytic Society on the grounds that his procedures were too unorthodox (he didn't see his patients five times a week for fifty-minute hours and insisted, moreover, upon giving them advice). Nonetheless, he spoke up for psychoanalysis as a way of untangling problems of the mind-body relationship, and at Bellevue nurtured a whole flock of younger psychiatrists who went on to become famous in their own right, such as John Frosch, founding editor in 1953 of *the Journal of the American Psychoanalytic Association.*[89] It might be argued that Schilder was not a true refugee, having left in the late 1920s by his own choice. Yet his departure was owing to the refusal of the anti-Semitic professor of psychiatry, Julius Wagner-Jauregg, to advance him,[90] and there is no question that Schilder, like Nunberg, Alexander, and the others who emigrated, while not being, strictly speaking, refugees, felt the bite of anti-Semitism.

The newcomers were accepted as stars. Else Pappenheim of Vienna was 28 when she arrived in 1939 at the Phipps Clinic in Baltimore. Back home, she had been a mere psychiatry resident and moreover was still in the middle of her own training analysis. But at Hopkins, she soon became an object of wonder. "As a physician from Vienna, I was accepted professionally immediately, even to the point of being hero-worshipped," she said.[91] Such was the prestige of Vienna-style psychoanalysis at the Phipps that, at rounds, Meyer would always ask New Yorker Joseph Wortis, who had spent four

months in Vienna being analyzed by Freud, “And what would Freud have thought about that?” Wortis confided to Pappenheim that he always bluffed some kind of answer.[92]

The refugee analysts played a particularly important role in alliance with the younger generation of Americans in forcing the training institutes that had budded in the 1930s to ensure the orthodox teaching of a given body of doctrine, in contrast to the eclectic, haphazard, and often eccentric training that had previously prevailed. What became orthodoxy in the United States was the “ego psychology” that Freud had first proposed in 1923 on the structure of the psyche (in which the ego and the id struggle over unbearable ideas). Freud’s daughter Anna became the chief standard-bearer of ego psychology.

In the United States, ego psychology broke away from the sexual doctrines of id psychology to emphasize the adaptation of the adult patient to social demands. It was a doctrine that suited the American analysts as progressive in spirit and practical in the prospect of improving patients’ lives, as opposed to Freud’s own pessimistic views about the inevitability of repression in civilized life. Ego psychology was also a particular gift that the younger emigré analysts were able to bring in their luggage. Ego-psychologist Heinz Hartmann, one of the last candidates to receive a training analysis from Freud himself in Vienna, became known as “the American prime minister of analysis.” Ernst Kris and Rudolph Loewenstein joined with Hartmann in becoming a triumvirate reigning over ego psychology in the 1950s and 1960s.[93] Thus the young Americans who were just founding training institutes, such as Ralph Kaufman and Ives Hendrick in Boston or Lawrence Kubie and Bertram Lewin in New York, made common cause with the refugee Central Europeans to capture psychoanalytic training.[94]

The European newcomers also overwhelmed the Yankees by virtue of their sheer prestige. The writings of none of the American analysts were known abroad. By contrast, many of the new arrivals from Central Europe were international celebrities. When in 1966, Arnold Rogow asked a sample of 31 psychoanalysts to name “the most outstanding living psychiatrists and psychoanalysts,” six of the top seven names were European refugees[95]: Anna Freud was in first place, followed closely by Heinz Hartmann and Erik Erikson. American-born Phyllis Greenacre was number four, known for her work on the psychoanalytic study of the child. Fleshing out the list were Rudolph Loewenstein, who had studied medicine and psychoanalysis in

Berlin in the early 1920s, ending up, after a 13-year interlude in Paris, in a Fifth Avenue practice in New York, where he wrote about ego psychology; the Viennese René Spitz, who came to New York via Paris and became known as an authority on nurturing children; and Robert Waelder, a nonmedical Viennese analyst who had settled in Philadelphia and became a popular interpreter of Freud. When in 1980, the members of the New York, Boston, and San Francisco psychoanalytic institutes were asked to identify the profession's leaders, six of the seven top figures were again refugee analysts, the list being similar to the preceding one.[96]

Thus the handful of refugee analysts from Europe had succeeded in imposing themselves as the leaders of American psychoanalysis. "These analysts formed a kind of bodyguard around an imaginary Freud," wrote Martin Grotjahn many years later—himself a Berlin psychiatrist and analyst who in flight had landed initially at the Menninger Clinic in Topeka. "They tried to keep psychoanalytic theory, technique, therapy, and training unchanged for years to come." Instead of "the relaxed and free debating atmosphere of the psychoanalytic coffeehouses in Berlin," Grotjahn found a "frighteningly standardized" American product,[97] one little capable of change or adaptation to scientific findings about the nature of cognition and psychotherapy. By helping to turn analysis into a temple devoted to the last remaining dinosaur ideology of the nineteenth century, the refugee analysts unwittingly guaranteed that the temple would soon come crashing down.

Triumph

But first intervened the years of triumph from the late 1940s to the late 1960s. As Seymour Sarason, who had observed these events from a catbird seat in the Yale psychology department, looked back in retrospect, "American psychiatry before World War II was biological psychiatry and within a few years after the war it was largely a psychoanalytical psychiatry."[98] The rising influence of psychoanalysis was expressed partly in numbers. The American Psychoanalytic Association grew from 92 members in 1932 to around 1,300 in 1968, at which time there was roughly one psychoanalyst for every 13 psychiatrists.[99] By the 1960s, in what Lewis Coser calls "the golden age of psychoanalysis in America," there were 20 training institutes, 29 local

societies,[100] and a climate of opinion in which psychoanalysis was the therapy of choice for dysphoric members of the middle classes. This boiling growth was fueled partly by the GI Bill of Rights, which paid for psychiatrists' psychoanalytic training as long as it took place in institutes. But the real motor of growth was the analysts' unremitting mandate to extend Freud's domain to all of psychiatry and to the American public.[101]

Starting from the early 1940s, psychoanalysis began taking over the prestigious psychiatry chairs and university departments. Each new conquest was celebrated as a triumph within the movement. Because New York—which in 1940 contained over a third of the analysts in the entire country[102]—was the movement's epicenter, the takeover began earliest and penetrated most extensively there. New York saw the foundation of a series of training institutes. In 1941, Karen Horney left the New York Psychoanalytic Society, taking a group of adherents with her to found the rival American Institute for Psychoanalysis. In 1942, this new institute was able to insert its training program in the New York Medical College.[103] In June 1942, a second group of schismatics from the New York Psychoanalytic Society led by Sandor Rado founded yet another rival organization, the Association for Psychoanalytic Medicine. Rado's people then pulled off a real coup in 1944 by persuading the department of psychiatry of Columbia University to agree to the establishment of a psychoanalytic training institute there, the first in the United States situated within a major institution.[104] After the American Psychoanalytic Association changed its charter in 1946, permitting the establishment of more than one training institute in a given city, the Psychoanalytic Clinic of Columbia University became a powerful bastion of university-taught psychoanalysis.

After the war, the takeover of university departments began outside New York. At Yale University, the Institute of Human Relations had long been, in Sarason's phrase, a "buzzing center of psychoanalytic thinking."[105] Then in 1948, a group of young psychoanalytically oriented psychiatrists ousted Eugen Kahn, a student of Kraepelin from Munich, who had come to the United States in 1930 and whose special research interest was the genetics of schizophrenia. Frederick ("Fritz") Redlich, an analyst from Vienna, became chairman of the psychiatry department, situated in the building of the Institute of Human Relations. At this point, virtually every prestigious chair of psychiatry in the country fell into the hands of an analyst: Kenneth Appel became chair of psychiatry at the University of Pennsylvania in 1953;

Royden Astley (a member of the Philadelphia society) got the professorship at Pittsburgh in 1956; in that same year Western Reserve in Cleveland asked Maurits and Anny Katan, also members of the Philadelphia society, to run a new psychoanalytic training institute within the school of medicine (Maurits became the professor of psychiatry).[106] The list could be extended at length.

The emigré analysts, with memories still live of their craft being spurned by the European professoriate, were astonished at these developments. Edith Weigert had been a resident physician at Ernst Simmel's psychoanalytic sanatorium in Berlin in the early 1930s and fled Germany in 1938. As she reported to her German colleagues in 1953 (now that she was chair of the Washington Psychoanalytic Institute), "Psychiatry chairs in outstanding universities are increasingly being filled with psychoanalysts or with psychiatrists who recognize analysis." She might have had in mind people such as John Whitehorn at neighboring Johns Hopkins University, who was sympathetic to analysis though not himself a psychoanalyst. He had replaced Meyer in 1941, and it was under Whitehorn that the department of psychiatry at Hopkins got its first full-fledged analyst, Theodore Lidz, who began his psychoanalytic training in 1947.

"Psychoanalysis in the United States," continued Weigert, "has not descended to the status of serving maid of psychiatry, as Freud feared it might ... but instead has tended to become psychiatry's highly respected pathfinder."[107] Swiss psychiatrist Henri Ellenberger, who in 1953 had taken a post at the Menninger Clinic in Topeka (and who in 1970 would establish for himself a towering reputation as a historian of psychiatry), noted there were many areas in which American psychiatrists were not strong: They were poor in classifying illness (nosology), poor in "phenomenology" (the patient's actual experience of symptoms), poor in "constitutional" approaches, nil in psychiatric genetics. But in psychoanalysis they were tops. "Of all the countries of the world," wrote Henri Ellenberger in 1955, "America is the first to have adopted a dynamic psychiatry [psychoanalysis] as its leading psychiatric trend."[108]

As the analysts reached for the university chairs, they grasped for control of the discipline of psychiatry as a whole. In organizational terms, psychoanalysts and analysis-sympathizers took over much of the apparatus of the American Psychiatric Association. In the late 1940s and early 1950s the organization had a spate of presidents who themselves were either analysts, such as William Menninger, the younger of the two Menninger brothers, or

strongly sympathetic to it, such as Whitehorn. Then in the 1960s, the APA presidents became uniformly analysts or members of organizations closely affiliated with analysis.[109]

One such organization was the Group for the Advancement of Psychiatry (GAP), founded in 1946 at a meeting of the American Psychiatric Association by a group of Young Turks whose leader was William Menninger. Its purpose was to bring social activism and encouragement for psychoanalysis to the parent Association. For example, in 1950 a GAP manifesto stated that social reality could influence anxiety (true enough) and that such reality "also exercises a selective influence on the choice of defense against anxiety (projection, reaction-formation, symptom formation, sublimation, and others)."[110] This kind of formulation was mother's milk to a Freudian, and GAP took a strong position in favor of psychoanalysis. Of 177 GAP members in 1948, 30 percent held posts in the American Psychiatric Association. This was three-quarters of all the committee posts in the APA.[111]

Yet the surest route to controlling the discipline of psychiatry was not to capture its professional organization but rather take charge of psychiatric training. Psychoanalysis was able to exercise such an enormous influence on American psychiatry not because the number of full-time analysts was so large but because analysts wrote the textbooks, staffed the university departments, and sat on the examination boards. In 1953, of 7,000 U.S. psychiatrists only about 500 were psychoanalysts.[112] Yet the influence of analysis ranged far beyond these numbers. From the 1940s to the 1970s, American psychiatrists, generally speaking, were not psychoanalysts as such but were psychoanalytically *oriented*.

The infiltration of psychiatry by analysis accelerated in 1952 with a joint report of the American Psychiatric Association and the national body of medical educators, the Association of American Medical Colleges. Everybody now agrees, the report said, that a competent psychiatrist has to understand "the principles of psychodynamics," including "Freudian concepts." There simply were not enough psychoanalytic training institutes to go around and so ways must be found to infiltrate psychoanalytic knowledge into the graduate curriculum. "... Such knowledge finds its way into the content of residency training programs through many different practices—supervised therapy, analytically oriented case discussions.... Better scientific communication about ... the supervision and structuring of so-called

‘psychoanalytically oriented therapy’ is greatly to be desired.”[113] As Karl Menninger noted in 1953, “Gradually the dynamic concepts have gained complete supremacy.”[114] (Dynamic was a code word for psychoanalytic.)

Meanwhile, the analysts had convinced the public that Freud’s teachings contained the secret of happiness. Market forces therefore would also contribute to the new supremacy of analysis within psychiatry. Residents demanded psychoanalytic training simply because the public now expected psychoanalysis and a good income was to be made in satisfying that demand. As a 1951 survey of 42 psychiatry residents at the College of Medicine of the State University of New York and Columbia concluded, “Since training in a psychoanalytic institute is considered the stamp of being a ‘first-class’ citizen, it is inevitable that none wants to be considered a ‘second-class’ one.”[115]

Yet it was not just “the New Yorker Syndrome,” as this hunger for analysis would later be called.[116] Residents everywhere desired psychoanalytic training. GAP was writing for the whole country when it said in 1955, “At present the prestige value of being a psychoanalyst is high, and it appears to offer greater financial rewards.” Of 165 psychiatry residents polled by GAP, “All ... indicate their desire for personal psychoanalysis and psychoanalytic training.” Of those residents, 20 percent were currently having a training analysis, and 26 percent were receiving psychoanalytic training in their residency.[117]

Established analysts were just as avid to teach Freud as residents were to learn. Of a sample of staff members polled in 1951 in various psychiatry departments, 56 percent had received formal psychoanalytic training including a personal analysis (and a further 11 percent had had a personal analysis without formal training, a practice the national psychoanalytic association thundered against for years).[118] When in 1955 the GAP surveyed 14 graduate programs in psychiatry, “all indicate that their training program is based on psychodynamic theory.”[119]

What were the most influential books in teaching psychiatry around 1965? In more than half of all programs, the basic list consisted of 17 titles, almost all of which were works of psychoanalysis ranging in alphabetical order from August Aichhorn’s *Wayward Youth* to Gregory Zilboorg’s *History of Medical Psychology*.[120]

The payoff from this approach was that by 1966, a third of American

psychiatrists had received some kind of psychoanalytic training, and 67 percent said that they employed “the dynamic approach” with their patients.[121] By the mid-1960s, psychiatry had come in the mind of the American public to mean psychoanalysis. The seizure of power was virtually complete.

The most baleful aspect of the takeover of psychiatry by psychoanalysis was the analysts’ ambition to extend their theories to the diagnosis and treatment of psychotic illness. If one is going to take over psychiatry, one has to have something to say about psychosis which, until the 1920s, was the discipline’s core interest. From the very beginning therefore, analysis had toyed with explanations and treatments of schizophrenia, mania, and psychotic depression. And although Freud publicly discouraged his followers from burning their fingers on psychotic patients, in private he was more permissive, and members of his inner circle had no compunctions about taking on the major psychiatric illnesses.

In 1908, Ferenczi became keen on treating Frau M., who suffered from paranoia. Yet first Ferenczi sought the Professor’s opinion: Did Freud think she needed to be treated in an institution, or would an outpatient basis suffice? Freud replied, “I’ve seen Frau M. She has frank paranoia, and probably is beyond the border of treatability; still, you can have a go at it and learn from her at all events. Her brother-in-law, who is accompanying her and is a doctor, is an ass. He will probably advise something other than what I’ve proposed. I demanded that she enter the Budapest [private] asylum and there let herself be treated by you.”[122] Freud’s disciple Paul Federn proposed a formal psychoanalytic assault on psychosis, becoming known as the great analytic advocate of reaching beyond neurotic patients.[123] Yet these European efforts to apply psychoanalysis to psychosis usually petered out after a trial or two, and in Europe analysis remained *grosso modo* an approach to the neuroses.

It was in the United States that psychoanalysts made their strongest attempts to treat psychosis. The seminal figure here is Adolf Meyer, an early member of the American Psychoanalytic Association and a large presence in the local Washington-Baltimore society. Although Meyer claimed to practice “objective psychobiology” and not psychoanalysis, he often sent patients to analysts. It is characteristic of Meyer’s own constitutional state of confusion that he clarified his stance toward psychoanalysis thus: “I feel but rarely the urge to go far ahead of the attitude of inquiry to a need of finality which will take care of its own lack of necessity.”[124] (At the same time that Meyer was

sending some patients to psychoanalysts, he was referring others to Henry Cotton at the Trenton State Hospital to have their teeth or colons removed for madness.) As early as 1909, Meyer toyed with psychoanalysis as a way of understanding schizophrenia, and such students of Meyer's as Edward Kempf (who left Meyer's department in 1914 to become a full-time psychotherapist at the government asylum, St. Elizabeths Hospital, in Washington) and C. Macfie Campbell (later director of the Boston Psychopathic Hospital) were ardent fans of applying analysis to patients with psychotic illness.[125]

Thanks to Meyer, the tradition of using psychoanalysis on patients with major mental illnesses rooted itself firmly in the Washington-Baltimore area. It was here that two private nervous clinics—Chestnut Lodge in Rockville, Maryland (founded in 1910 by Ernest Luther Bullard) and the Sheppard and Enoch Pratt Hospital in Towson, Maryland (opened in 1891 as the Sheppard Asylum), became the American flagship hospitals for applying psychoanalysis to gravely ill patients. In December 1922, Harry Stack Sullivan, perhaps the most famous figure in the psychoanalytic treatment of the psychoses, arrived at the Sheppard.

Sullivan had received his analysis training in 1916 and 1917 from Clara Thompson, one of the strong female figures in the history of American psychoanalysis and a confederate of Horney. He had become interested in psychosis while on staff at St. Elizabeths Hospital under the tutelage of director William Alanson White, one of the best-known American psychiatrists to acquire an early interest in analysis. Sullivan came to the Sheppard with the understanding that he would have a free hand. He created a special six-bed ward where he achieved quite good results with patients he had diagnosed as "schizophrenic." (These were the days when American psychiatrists used the diagnosis of schizophrenia far more frequently than their colleagues elsewhere.) In Sullivan's view, schizophrenia was an unsuccessful reaction to anxiety. He was known for the hours of sympathetic care he would give his patients, many of whom responded gratefully to the attention.[126] Although he was not a strictly orthodox analyst, he nonetheless became the spark plug in touching off interest in the psychoses within American psychoanalysis in general.

In 1930 Sullivan left the Sheppard, establishing himself in private practice in New York and Washington. Later in the 1930s, he would also found his own school of psychiatry and his own journal. Midst all this activity, large crews of psychoanalytically oriented psychiatrists and psychoanalysts began

mustering at the Sheppard and at Chestnut Lodge (by 1938, for example, six analysts from Chestnut Lodge alone belonged to the Washington-Baltimore Psychoanalytic Society).[127]

The situation took a new turn in 1935 with the arrival at Chestnut Lodge of Frieda Fromm-Reichmann, a refugee German analyst with a long history of private-clinic work. (Frieda Reichmann was married briefly to analyst Eric Fromm.) She became influenced to some extent by Sullivan's ideas, given that he was the chief figure in the Washington-Baltimore region. Yet she went a step farther.[128] Fromm-Reichmann did not just believe in standard psychoanalytic notions about anxiety and such as the cause of schizophrenia. She knew that it was the work of the mother in particular. Generations of American mothers had to suffer unwarranted reproaches as "schizophrenogenic mothers"—to use Fromm-Reichmann's notorious phrase—after her writings on this subject began to appear in 1948. What was the problem in schizophrenia? "The schizophrenic is painfully distrustful and resentful of other people," said Fromm-Reichmann, "due to the severe early warp and rejection he encountered in important people of his infancy and childhood, as a rule, mainly in a schizophrenogenic mother."[129] There one had it. The mother must not leave the side of the child. The psychoanalytic incursion into psychiatry, much like Napoleon's invasion of Russia, had reached its furthest point.

Far from being marginalized, Sullivan's and Fromm-Reichmann's teachings on the causes and treatment of psychotic illness became widely adopted in psychiatry. Fromm-Reichmann's schizophrenogenic mother became the basis of "family systems theory" in the treatment of schizophrenia, and such therapists as Gregory Bateson at the Mental Health Research Institute in Menlo Park, California, postulated a complex "double bind" theory of the disease, in which the mother emerged as the sickest member of the family. "It became standard practice," wrote one scholar, "to believe that mothers were the cause of their children's psychosis."[130]

Beyond schizophrenia, analytic interpretations of other psychotic illnesses became standard dogma within American psychiatry. The cause of mania? An oral triad of "a wish to eat, a wish to be eaten ... and a wish to go to sleep," said New York analyst Bertram Lewin in 1951.[131] Depression? "A despairing cry for love," said Sandor Rado. The ego tries to punish itself to forestall punishment by the parent.[132] Paranoia? It arises in the first six months of life, said London analyst Melanie Klein (who had come to England

from Budapest via Berlin), as the child spits out the mother's milk, fearing the mother will revenge herself because of his hatred of her.[133] When in 1958 the American Psychoanalytic Association organized a program to import the teaching of these doctrines into mental hospitals, the campaign by psychoanalysis to take control of psychiatry had shattered the last resistances.[134] A psychiatry resident from abroad recalled the scene at Delaware State Hospital once the analysts had taken control of it: "The training in individual and group psychotherapy was given over to psychiatrists who came from the various university clinics in nearby Philadelphia. Their teaching model was psychoanalytic psychotherapy. It was urgently driven home that we should view institutional psychiatry merely as a brief transitional stage for us. We should begin our training analyses as soon as possible. Our ideal professional goal was doing psychoanalysis in private practice in combination with supervising training at one of the psychoanalytic institutes independent of a university department. From the viewpoint of the psychoanalytic theories of the 1940s, our daily therapeutic activities [at Delaware] were considered highly questionable. The somatic therapies, so we were told, were stopgaps. They concealed instead of uncovering. Ordering a sedative for even an agitated psychotic patient was not therapeutic for the patient but considered an anxiety reaction on the part of the doctor. Whoever expressed the slightest doubt about psychoanalytic interpretations, making reference to other theories, was considered to be neurotically disabled from overcoming his resistances."[135] Into these analytic hands, the discipline of psychiatry had been delivered.

What is ironic, given the attempt of analysts to dominate psychiatry, is their contempt for the precise diagnosis of psychiatric illness, the intellectual core of psychiatry over the previous hundred years. Psychoanalysts ridiculed attempts to divide mental illness into specific categories, as Kraepelin had done on the basis of course and outcome. According to Karl Menninger in 1956, "The old Kraepelinian terms have largely disappeared." [136] But not just the Kraepelinian terms: Gone was any sense of well-defined disease entities. Menninger: "Gone forever is the notion that the mentally ill person is an exception. It is now accepted that most people have some degree of mental illness at some time, and many of them have a degree of mental illness most of the time."[137] We are all, in other words, a little bit schizophrenic or a little bit manic-depressive (an assertion that would rule out a genetic basis for those illnesses). Menninger once told refugee psychiatrist Lothar

Kalinowsky, who had a strong background in organic psychiatry, “I consider you to be an intelligent individual and cannot understand why you are interested in questions of classification.”[138]

The Menninger family as seen in 1951. In the minute the father, Charles F. Menninger, who along with his son Karl (on the right) founded the Menninger Diagnostic Clinic in Topeka, Kansas, in 1919. On the left is C. F.’s other son William, who helped found the Group for the Advancement of Psychiatry in 1946 (courtesy of the Menninger Clinic).

Behind the analysts’ indifference to the fine Kraepelinian distinctions between affective and nonaffective psychosis was the conviction that only one form of psychiatric illness existed, and that this form exhibited mere quantitative differences on the basis of how severely one had failed to adapt to the environment. Since there was only a slippery slope rather than an absolute line between the ill and the well, the analysts considered it pointless to talk of curing “disease.” We were all the walking wounded, normal neurotics. Rather, the purpose of psychiatry was to understand the meaning of the symptom and “undo its psychogenic cause”—in the words of one observer—“rather than manipulate the symptom directly through medication, suggestion, etc.”[139]

The analysts' indifference to determining what "disease" the patient suffered infiltrated all of American psychiatry. "In Europe," reflected Ellenberger, "people go to the psychiatrist because of a symptom, in America because of a *problem*."[140] Symptoms came from disease, problems from society. The whole concept of disease implied brain lesions, disorders of neurotransmitters, genetic loading, and the like. If psychiatric illness were psychogenic, the product of anomalous childhood socialization exacerbated by the inability to adapt, psychiatric disease did not actually exist unless one had neurosyphilis. Continued Ellenberger, "I remember the consternation of a German psychiatrist hearing that certain American colleagues consider manic-depressive psychosis as a form of schizophrenia; for him this was about as fantastic as the assumption that a camel is a subspecies of the elephant. On the other hand, Americans can hardly understand the strenuous efforts of the Europeans to isolate and individualize mental diseases; they have the impression that the Europeans are satisfied with merely labeling the disease. Nosologic [classificatory] discussion of whether paranoia is a subform of schizophrenia, or a specific disease, appears as ridiculous to them as the Middle Ages' theological controversies on the sex of angels."[141]

This incuriosity about the nature of illness meant that when the analytically trained psychiatrists encountered patients with serious disorders, they were baffled. Lawrence Kubie and Sandor Rado would refer patients to the psychosurgery unit that Columbia University's Department of Neurology ran in common with the New Jersey State Hospital at Greystone Park (the "Columbia-Greystone Project") with a diagnosis of "pseudoneurotic schizophrenia." The meaning: We know something serious is wrong with these people but we can't tell what. These referrals were not innocuous for the patients because they may have subsequently undergone lobotomy, surgical excision of brain matter.[142]

The flip side of misdiagnosing major psychiatric disorder was in the inappropriate application of psychobabble to individuals who were well but different in background from the analyst. Here is a group of psychoanalytically oriented psychiatrists in 1955 in Cleveland at the bedside of a 75-year-old black laborer who had undergone a prostate operation, and whose behavior in the hospital had apparently prompted the surgeons to request a psychiatry consultation. Although the psychiatrists found the man "normal," which they put in quotation marks, they nonetheless found pathological his "pride in being 'the man who worked' in his family," as well

as his belief in getting up as soon as possible “because lying around weakened a man, and in his concern about having enough ‘pep’ postoperatively.” They took all these as “evidences of pseudomasculinity.” [143] The message: Black laborers who want to retain their pride are being pseudomasculine (and would benefit from psychoanalysis if they could afford it).

In any event, analysts for the most part had little interest in treating black laborers. Analytic practices had a much larger proportion of professionals, executives, and the like than did practices that used a variety of therapies excluding analysis.[144] The psychiatry of those years distinguished between “good” patients and “bad.” Good were those who, as Herman van Praag—later head of psychiatry at Albert Einstein College of Medicine—put it, were “fairly young, quite intelligent, and introspective.” They tended to be educated and middle class. Bad were the seriously ill and chronically disabled, the schizophrenics and the addicts, the patients who were often uneducated and poor. “In other words, the patient had to fit the treatment, rather than the treatment being adjusted to the patient.” [145] For thoughtful clinicians such as van Praag, the discipline of psychiatry had become an embarrassment, a mockery of medicine.

By the 1960s, psychoanalysis was at the apogee of its success within psychiatry. Although analysts numbered only about 10 percent of all psychiatrists,[146] the influence of psychoanalysis reached into most of the private practices in the country. The biological psychiatrists, in contrast, had been limited to unglamorous posts in state hospitals. Analysts were being consulted by government agencies and the Congress. Being “shrunk” became the ne plus ultra of upper-middle-class American life.

Under the influence of psychoanalysis, American psychiatry completed its long march from the asylum to Main Street. Analysis could only be done in private practice, midst the hubbub of middle-class life. In 1917, only about 8 percent of American psychiatrists were in private practice, a figure that had increased only to 31 percent by 1933.[147] In 1941, psychiatrists either entirely in private practice or in institutions with a private office on the side numbered only 38 percent of the total.[148]

The balance then shifted dramatically during the heyday of psychoanalysis. By 1970, at least 66 percent of all American psychiatrists were in private practice, in reality doubtless more because many of those with appointments

in hospitals and universities also had private offices on the side.[149] Of those psychiatrists who in 1941 had been entirely in hospitals and asylums, by 1962 half had gone over to private practice. (Interestingly, with the decline of analysis in the 1970s and beyond, the percentage of psychiatrists in institutional settings would again rise: 11 percent of the total in 1988 were in private, free-standing psychiatric hospitals.) [150] The victory of the psychotherapeutic model seeded psychiatrists throughout the middle classes of the big cities. An analytically oriented therapist would be almost as close at hand as an optician or a lawyer in case of trouble.

This apparent indispensability in the crises of middle-class life brought to psychoanalytically oriented psychiatry its own arrogance. Its lieutenants dealt swiftly with doubters by giving them psychiatric diagnoses or simply by shouting them out of court, as Hopkins child psychiatrist Leon Eisenberg discovered in 1962 when he ventured a few critical remarks about the scientific nature of psychoanalysis at a medical educators' meeting. "There was a veritable stampede of Department Chairmen to the floor microphones.... Just about every eminent figure present rose to defend the primacy of psychoanalysis as 'the basic science' of psychiatry."[151] When in 1964 *Fact* magazine polled 2,400 psychiatrists about whether Senator Barry Goldwater, a Republican, could safety be elected President over Lyndon Johnson, 1,189 pronounced Goldwater "psychologically unfit to be President."[152] The hubris of the analysts was extraordinary. Shortly, it would all come crashing down.

Psychoanalysis and the American Jews

It is impossible to understand the extraordinary rise of psychoanalysis in terms of "internalist" perspectives alone, meaning developments within a discipline such as the evolution of ideas or the succession of personalities. So hallucinatory did the sexual perspectives of psychoanalysis seem that its ascent and triumph could not possibly be explained merely on the basis of sound ideas driving out invalid ones. Events in the external world helped make possible its ascendancy, granting it a momentum that its internal dynamic would never have achieved. These are "externalist" perspectives on the history of psychiatry. Various such factors made a difference, from a

rising affluence among the middle classes that permitted such personalized care, to the desire for introspection that humanistically educated people often bear away from the university. Some observers have commented on the lack of authority of the American professoriate, which let academic objections to psychoanalysis be swept aside, whereas in Europe the professoriate were able to hold up the onmarch of analysis.[153] One historian sees the "hothouse" nature of American family life creating a kind of "oepidal" nest into which a belief in analysis could burrow.[154] I want to call attention to yet another external factor, not because it is necessarily the most important one in the rise and fall of analysis but because it runs like a silver thread from the beginning of the story to the end: It is the history of the Jews in Europe and North America.

From the viewpoint of the history of psychiatry, the vicissitudes of the Jews in the Old World and New were a matter of capital importance. The common theme linking the misadventures of psychoanalysis on both sides of the Atlantic was the desire of recently acculturated middle-class Jews for some symbol of collective affirmation. Although Freud sought mightily to downplay any kind of ethnic specificity in psychoanalysis, the subtext of Freud and his followers to the non-Jewish charter culture was: We Jews have given this precious gift to modern civilization.

Why would Jews need such a symbol any more than any other ethnic group? In the history of modern times, Jewish people have had to endure not just one but two great shocks. Every people that undertakes the long journey from small-scale life in the traditional village to middle-class life in the big city undergoes one major shock: the shock of assimilation and integration, the psychological upheaval that goes with newness of arrival. In their move from shtetl life in the small towns of eighteenth-century Poland and the Ukraine to such bustling cities as Berlin, Frankfurt, and Vienna, the Jews underwent this shock just as everybody else did. But then a second shock lay in store for the Jews, the Holocaust, and the forced transplantation of hundreds of thousands of individuals who themselves had only recently become middle-class, from a comfortable and bourgeois European existence to the nightmare of scrambling for a passage to America. This second shock was experienced by no other cultural group.[155] It profoundly shaped the desire of the American Jews for some kind of a special symbol of self-affirmation, a collective badge of pride in the chaos of the living city. That symbol, I argue, was psychoanalysis.

At the turn of the twentieth century, the Jews of Central Europe were experiencing the cultural confusion of a massive deracination. Between the 1860s and 1900, countless numbers of people were torn from the ghettoes and shtetls of Eastern Europe, without becoming as yet newly rooted among the middle classes of the West. Many of the Jews of Berlin and Vienna had left their religion behind and were rapidly trying to assimilate by changing their names and by converting to Protestantism (less so to Catholicism). Yet despite their best intentions, despite their knowledge of the plays of Schiller and of the refinements of the German language, they encountered a baffling wall of anti-Semitism.

There was something about psychoanalysis that made it, according to historian John Cuddihy, a "plausible ideology for [a] decolonizing people."[156] Jewish patients with psychoneurosis were therefore drawn to it. Perhaps psychoanalysis was seized upon because it extended the possibility of finding one's identity from within, as opposed to the external signposts that orthodox Judaism offered. And it may have appealed to Jewish women in particular. Perhaps these cloistered but well read and highly curious women —members of a "middle-class drenched in spirit" in the words of Viennese novelist Robert Musil—were simply more self-reflective, more psychologically minded that the women of the non-Jewish lower-middle classes below them who worked alongside their husbands in shops, or the women of the nobility above, busy with the social whirl of the salon.[157] Or perhaps Jewish men and women alike adored psychoanalysis because it was "our thing." In any event, psychoanalysis in the early days had a very specific social address.

It was above all among the middle-class Jews of Berlin, Budapest, and Vienna that psychoanalysis proved such a hit. Historian Steven Beller finds the Jews of Vienna, as outsiders, using psychoanalysis to "make a political attack on Viennese society by an alliance of scientific rationality with instinct" against the city's traditional sensual baroque culture.[158] In Budapest, there were descriptions of psychoanalysis in the Jewish quarter, the Leopoldstadt, as an almost "incomprehensible and impenetrable secret doctrine or ceremony...."

Historian Paul Harmat concludes, "Psychoanalysis was most popular among enlightened Jewish circles as a result of their minority situation." [159] Of course, non-Jews had recourse to analysis as well. Yet among patients, there seems to have been a kind of Jewish tropism.

The analysts themselves also tended heavily to be Jewish, and many of them assumed that Jewishness helped one to appreciate Freud's wisdom fully. As Freud said in 1908 to the Berlin analyst Karl Abraham, on the occasion of a malentendu with Carl Jung (then one of the few non-Jews in the movement), "Please be tolerant, and don't forget that it is actually easier for you than for Jung to follow my ideas ... because you stand closer to me as a result of racial affinity, while he, as a Christian and son of a pastor, finds the way to me only in the face of great inner resistance." On another occasion, Freud reassured Abraham, "May I say that what attracts me to you are our related, Jewish characteristics. We understand each other."[160] Freud's inner circle was almost entirely Jewish, and Ferenczi said to Freud of the one non-Jewish member, the Londoner Ernest Jones, "It has seldom been so clear to me as now what a psychological advantage it signifies to be born a Jew.... you must keep Jones constantly under your eye and cut off his line of retreat."[161]

Within the middle-class Jewish public, psychoanalysis became signposted as belonging to some larger Jewish worldview. Humorist Salomo Friedländer, writing in the 1920s under the pseudonym "Mynona," made analysis the portal through which Christians who wanted to convert to "true Judaism" must pass. In one tale Friedländer allows the wildly anti-Semitic Count Reschock to fall in love with the beautiful Rebecka Gold-Isak. Losing his bearings completely, the Count decides to convert to Judaism to win his prize. Rebecka insists that he must become truly Jewish before she will accept him. The Count's first step on the path of a Jewish identity is an analysis with Professor Freud. "This destroyer of fig-leaves," as Friedländer termed Freud, "robbed the noble Reschok soul of its protective coat with such anatomical certainty that the Count fell with a cry into the arms of his alarmed servant." (Reschock goes on to have a famous surgeon convert him from a blonde Prussian warrior into a "Jewish Torahstudent.") [162] Jewish and non-Jewish readers alike found the Friedländer fable delicious, yet accepted implicitly its premise that psychoanalysis was identified with Judaism. If psychoanalysis is written as a history of ideas, these social themes are unimportant. But if we try to understand its rise and decline as a movement, the singular tropism that many Jews felt toward analysis, both as doctors and patients, is of considerable significance.

With the passage of time, in Europe at least, psychoanalysis lost its Jewish stamp. Although it had originated among the Jews of Vienna and Berlin, as it

developed, it ceased to be their property. There was certainly no Jewish tropism among the chief physicians of the many private clinics that offered psychoanalysis. And in Switzerland and England, psychoanalysis was known to be a specifically non-Jewish affair. As Swiss psychiatrist Max Muller commented of the 1920s, "It was characteristic of the psychoanalytic movement in Switzerland that, unlike other countries, it did not consist predominantly or almost exclusively of Jewish physicians and lay-analysts."[163] And the two most prominent advocates of analysis in Switzerland before 1914—Eugen Bleuler and Carl Jung—were if anything anti-Semitic. (It is perhaps indicative of the mood of the Bleuler household that, upon discovering that Viennese psychiatrist Erwin Stransky was Jewish, Bleuler's wife expressed great astonishment and said, "Well then you must at least have an aryan soul in you.")[164] Commenting on the plethora of Jews in psychoanalysis generally, Ernest Jones noted, with relief, that apart from the refugees, "in England ... only two analysts have been Jews."[165]

Before 1933, a number of Jewish physicians figured prominently among the opponents of analysis. In Central Europe, for example, Gustav Aschaffenburg, Adolf Friedländer, and Erwin Stransky—all Jewish—were among Freud's most outspoken enemies, as were Boston's Abraham Myerson and New York's Bernard Sachs in the New World. (Stransky took this to mean that no one could accuse the opponents of psychoanalysis of being anti-Semitic.[166]) It could be argued that by 1933 in Europe and even in America, psychoanalysis had shed its initially Jewish imprint.

After 1933, all this changed. As a movement, analysis in Europe was destroyed. Its main representatives who fled to the New World were Jews. For these battered and profoundly disoriented survivors, psychoanalysis became one of the Jewish accomplishments that could be presented to the host population as a ticket of entry. Among the refugee Jews, both physicians and nonphysicians—psychoanalysis became a badge of Jewish solidarity in the face of a population of Anglo-Saxons perceived to be racially hostile, psychologically insensitive, and culturally backward. Said Martin Grotjahn of his fellow emigré analysts, "Psychoanalysis symbolized for them the light of the Old Country to be carried to the New Country."[167] But it was a light that Jews had created, and in whose warmth they would bask for several decades.

The American Jews had not experienced the trauma of emigration. Yet they too had arrived as outsiders, and as psychoanalysis acquired new prestige in medicine after the Second World War many Jewish physicians

and patients alike were drawn to it as a symbol of collective self-affirmation: This is what we have created. By it we shall become better and in doing so bring enlightenment to others. After 1945, American Jews took on psychoanalysis as a kind of *mission civilisatrice,* a healing gift to all the world, which is not at all an overwrought formulation considering the prose with which Jewish analysts themselves described their mission to humanity. How things have changed for us, Franz Alexander assured his colleagues in 1953, "as soon as all that you professed is accepted and the world is asking you sincerely and avidly to explain the new truth. They turn to you now: 'Please tell us all about it. How does the new knowledge help us, how can we use it constructively to cure a neurotic or psychotic patient ... to alleviate social prejudice and international tension, and to prevent war.'"[168] Is it any wonder that Jews themselves would preferentially have recourse to this new knowledge?

Why had psychoanalysis spread so rapidly after World War II? asked psychologist Seymour Sarason. "Most analysts (and a significant portion of the psychiatrists who received training during the war years) were Jewish. For them, Hitler and fascism were not abstractions but threats to existence. And for them, Freud represented a Moses-like figure whose contributions had opened up new vistas about the nature of humans...." [169] For Sarason and Alexander, Jews were a gifted but marginal population, still ill at ease and unintegrated.

Surveys establish the extent to which Jewish physicians predominated in the practice of psychoanalysis. In 1959, two researchers drew up a profile of psychiatrists who believed in psychoanalysis: Eighty percent of them were of Jewish origin and tended to be upwardly mobile, insight-oriented, and deracinated (in contrast to the biologically oriented psychiatrists in the sample, who tended to be mainly Protestant). On a number of characteristics, the psychoanalytically inclined Jewish psychiatrists stood out from the non-Jews: They were agnostic, as opposed to the organically oriented Protestant psychiatrists who retained some shreds of their religious faith. They were more leftist, and they were more aware of the importance of social class, as opposed to the Protestant group who were somewhat embarrassed by the subject.[170] When Arnold Rogow quizzed a sample of 35 psychoanalysts and 149 nonanalyst psychiatrists in 1965, he found 26 percent of the analysts willing to declare they were Jewish; a further 17 percent were willing to say they had Jewish mothers; a third were unwilling to say anything about

religious affiliation. (By contrast, the figures for the nonanalyst psychiatrists were lower in all three categories.)[171] On the basis of these statistics, it is fair to infer that a majority of the practitioners of psychoanalysis were of Jewish origin though of course numerous non-Jews entered the field as well. How about patients? It seems to be the case that Jews overconsume most psychiatric services in proportion to their numbers in the population. This is certainly true of psychoanalysis. In Rogow's study, one third of the analysts said they had practices consisting heavily or overwhelmingly of Jews.[172] A variety of other studies revealed the same finding in other ways.[173] Most dramatic perhaps was a random, nationwide survey of the adult American population in 1976, which found that 59 percent of Jewish respondents had at some point in time received psychotherapy (in contrast to the non-Jewish help-seeking rate of 25 percent).[174] In other words, more than half of all American Jews had sought out psychotherapy at a time when psychotherapy was overwhelmingly psychoanalytically oriented. It is not stretching the facts to refer to psychoanalysis in the middle decades of the twentieth century as a kind of Jewish "our thing."

This is not the place for a comprehensive account of the decline of psychoanalysis (see pp. 305-313). Many factors were involved, such as the advent of effective new medications that obviated long sessions of therapy, a new model of psychiatric illness stressing neurogenesis rather than psychogenesis, and the rise of alternative systems of psychotherapy. Compared with these changes, developments within the Jewish community, previously an important bastion of psychoanalysis, do not bulk large. Yet since Jews are under discussion here, this might be the place to mention the role that the loss of a social base appears to have played in the plunging popularity of analysis. In my opinion the main source of this loss was the increasing social assimilation of the American Jews. They no longer required psychoanalysis as a badge of collective identity because they were no longer affirming themselves. Instead they were becoming like everyone else.

Consider the experience of "Deborah" in *I Never Promised You a Rose Garden,* Joanne Greenberg's semi-fictionalized account of her own psychotic illness, published in 1964:

> Greenberg was born in 1932 into families of recent immigrants on both sides. In 1944, around the age of 12, she started to fall ill with symptoms of what was undoubtedly a genuine psychosis. The Greenberg family

consulted Richard Frank, a New York psychoanalyst specializing in children, and when Joanne was sixteen, Frank had her admitted to Chestnut Lodge. He had encouraged the family to hold out until then because the Lodge did not admit children under that age and Frank evidently wanted his young patient to be treated psychoanalytically.[175]

The world the Greenberg family inhabited in those years was an intensely anti-Semitic one. In the novel, Deborah is cursed by her playmates at a "cruelly anti-Semitic" camp as a "stinking Jew." In their New York suburban home, the well-to-do Greenberg family (in the novel at least) had anti-Semitic slogans painted on the wall and rats flung at the porch. Is it any wonder that the Greenbergs encapsulated themselves in an almost entirely Jewish subculture. "In the place and time where Deborah was growing up, American Jews still fought the old battles that they had fled from in Europe only a few years earlier," Greenberg writes in the novel. And Deborah's friends were entirely Jewish: "I never knew anyone well who was not Jewish," she said, "and I never gave my last particle of trust to someone who wasn't Jewish."

What to do for Deborah? The mother had heard of psychoanalysis. She says to Deborah's psychiatrist at Chestnut Lodge: "They tell me that these illnesses are caused by a person's past and childhood. So all these days we've been thinking about the past. I've looked, and Jacob [her husband] has looked, and the whole family has thought and wondered...." And they can't figure out what they might have done to cause Deborah's illness.

But the psychiatrist knows. The psychiatrist was in fact Frieda Reichmann. It was Deborah's mother's fault. Reichmann ("Dr. Fried") coaxes from Deborah a memory (or suggests her into one) in which the mother abandoned her at an early age. The memory comes tumbling forth. "The rush of words ended and Dr. Fried smiled. 'It is as big, then, as abandonment and the going away of all love.'"[176]

New York in the early 1940s: Within this encapsulated little subculture what other therapy could have seemed appropriate for Jews than psychoanalysis? The psychiatrists were mainly Jewish analysts. The expensive private clinics such as Chestnut Lodge catered to wealthy Jewish families. The subculture itself understood analysis to be the only thinkable option because it was a therapy that had come over from the sparkling Jewish intelligentsia of Europe. (Dr. Fried explains that one of her European patients had died in Dachau.)[177] Deborah, the Greenbergs, and the Jewish middle

classes of the great American cities lived in a world permeated by psychoanalysis.

The breakout of the American Jews from this sequestered little existence occurred after the 1960s. Statistics on intermarriage are most revealing, because the chances of the children being raised as Jewish in a union where one partner is not Jewish are less than half as high as in unions where both partners are Jewish. Among Jews marrying before 1960, only 5 percent had chosen non-Jews; in 1960-1969, 12 percent; in 1970-1979, 19 percent; and in 1980-1989, 33 percent. The trend, in other words, was clearly toward intermarriage for young people. The same progression repeated itself on the basis of generation: For the first generation in the New World, intermarriage was 5 percent; for the 4th, 38 percent. Most unsettling of all, given the high levels of marital instability in the United States, was that in first marriages only 11 percent of the partners were non-Jewish; in second marriages 24 percent, and in third marriages 40 percent.[178] By 1990, as many as 52 percent of Jews were marrying outside the faith and fewer than half belonged to synagogues. The numbers offered a picture of a community in dissolution.[179]

This breakout from the Jewish subculture may have entailed the breakout from such former symbols of community solidarity as psychoanalysis. Recent data on the ethnicity of psychoanalytic patients are difficult to come by, yet one authority speculates that the percentage of Jews in the clientele of psychoanalysts has “probably declined.”[180] If Jews were like everybody else, they could participate in the same kind of psychiatric treatment that everybody else was receiving, and after the 1970s, it was not psychoanalysis.